



GUIDELINE

Acute behavioural disturbance assessment and management of patients with developmental disabilities

Scope (Staff):	Medical, Nursing, Allied Health
Scope (Area):	PCH – Inpatient admissions

Child Safe Organisation Statement of Commitment

CAHS commits to being a child safe organisation by applying the National Principles for Child Safe Organisations. This is a commitment to a strong culture supported by robust policies and procedures to reduce the likelihood of harm to children and young people.

This document should be read in conjunction with this [disclaimer](#).

If you have any feedback or comments to improve our clinical policies or our systems, please tell us by filling in the [feedback form](#).

Aims

- To provide guidance for staff in the assessment and management of patients with neurodevelopmental disabilities presenting to hospital for management of acute behavioural disturbance.
- To enable effective communication on all levels to ensure a safe and uncomplicated admission and timely discharge.

Scope

This guideline applies to all clinical staff involved in the treatment of children and adolescents with developmental disabilities with associated behavioural disturbances within Perth Children's Hospital.

Background

Recent studies have reported that children with neurodevelopmental disorders (e.g. Autism Spectrum Disorder - ASD) have higher rates of presentation to emergency departments and hospitalisation when compared to their typically developing peers. The nature of a child's ASD diagnosis and the associated difficulty with communication, sensory processing and social interaction can result in extreme anxiety, behavioural outbursts and self-injurious behaviour. This is particularly evident within a hospital environment with its inherent increased sensory stimulation (sounds, touch, visual), unpredictable routines and unfamiliar people.

A sudden or severe change in behaviour in a child with a neurodevelopmental condition can be a symptom of an underlying medical problem that is causing the child pain and discomfort related to¹:

- intercurrent physical problem or medical condition
- medication changes or side effects
- frustration and communication difficulties
- manifestation of another acute mental health condition including psychiatric episodes or anxiety

Examining a child in an acute state of arousal or agitation can be challenging for the clinician and for the child and family. To facilitate examination and diagnosis of a medical cause, it may be necessary to consider use of non-pharmacological techniques, anxiolytic medication, sedation or general anaesthesia to perform investigations, such as imaging / CT scan to avoid delays in treatment and / or discharge.

Risks

Delays in investigation, treatment and discharge planning can result in prolonged hospital stay and increased distress for the child and family.

Escalation of agitation or aggressive behaviour may result in injury to the patient, staff or others.

Key Principles

- Children and adolescents with developmental and cognitive disabilities have the right to be cared for with the same dignity and respect as any other child without a disability.²
- The hospital environment can be particularly challenging for children with neurodevelopmental conditions and failure to provide appropriate psychosocial support can exacerbate challenging behaviours.
- KKIND ('Keeping Kids in No Distress') should be consulted early in the admission for advice and ongoing support.
- An individualised plan of care is to be commenced on consideration of an admission and formulated in partnership with the child / adolescent's family, usual care workers and relevant specialist services. Refer to PCH Policy '[Engagement of Private Carer, Nurse or Therapist](#)'.
- Strategies for de-escalating acute agitation or harmful behaviours should be obtained from the patient's caregivers or community therapists and should commence with simple, non-pharmacological measures including:
 - Understanding of the child's means of communication
 - Sensory environmental modifications (for example maintaining a low stimulus environment where possible)
 - Individualised calming strategies and distraction (consult the individual's caregivers and Patient Profile / Hospital Passport (where available)).
 - Engagement in meaningful activity and relationship

- Investigate / exclude a medical cause with careful history taking and gentle physical examination.
- For pharmacological interventions refer to [Agitation & Arousal Guideline](#) (Clinical Practice Manual) and consider consultation with a psychiatrist / Child Development Service (CDS) clinician.
- The presence of the patient's usual care worker should also be considered where appropriate.
 - This must be in agreement with the child's legal guardian, the treating physician, Clinical Nurse Manager and the Director of Nursing.
 - The scope and delegation of care must be clearly documented in the patient's medical record.
- Facilitate communication, ongoing support and timely discharge planning at the earliest opportunity with the child and family's usual social support agencies. Inform community providers and Crisis Care (where appropriate) of an acute presentation.
- For any significant difficulties with admission, transfer or discharge early escalation to the hospital executive team is required.

PROCESS

On Presentation

- The Emergency Department (ED) evaluation of the patient with behaviour management issues requires an initial brief assessment aimed at determining the most likely cause of agitation and risk of injury or violence (see [Behaviour Problems - ED Guideline](#)).
- A comprehensive medical, psychiatric, psychosocial assessment will then be required.
- Wherever possible, information from family, carers and other service providers directly involved in the present care of the patient, should be sought. This information can aid in the diagnosis, assessment of risk, and influence management and discharge planning.
- Determine the child's capacities of comprehension and expressive language to reduce barriers associated with accessing health care and reduce distress associated with limited communication.
- Use of an interpreter for the culturally and linguistically diverse patient or their family / carer should be used where appropriate.
- Engage appropriate Aboriginal Liaison Officers and mental health professionals for patients from Aboriginal and Torres Strait Islander backgrounds.
- All staff involved in the care of a child with developmental disabilities should be cognisant of the individual needs of each child.

- Liaison with the child's family members, usual care workers and specialist agencies (e.g. NDIS Community Service Providers) is vital to determine baseline functioning and to help recognise stressful triggers and escalating behaviours and for implementation of effective strategies to help reduce anxiety.
- If the child is registered with NDIS, obtain consent to exchange information with NDIS. Seek the child's NDIS participant number and copies of the child's NDIS plan and positive behaviour support plans if the child's family are willing to share this information.
- Targeted consultations with service providers (e.g. Child Development Service) regarding support and treatment of the patient is advised.
- Detail should be sought on current medication plans, behaviour support plans, communication plans, aides and sensory considerations for the patient.

Pre-Admission

- Patients being considered for admission to an inpatient bed should be first discussed with the on-call medical Consultant or CAMHS Consultant. This discussion should involve consideration of:
 - physical, developmental and mental health issues, and
 - the most appropriate place (mental health ward or medical ward) taking into consideration:
 - staffing levels and skills required for optimum patient supervision (see [Nursing Care: Increased Level of, Observation and/or Supervision](#) policy) and
 - physical environment (potential for harm or injury to self and others; equipment in room and sensory triggers) for the duration of the patient stay
 - Following these considerations, in-reach may be required from either the medical team or CAMHS.
- If a suitable location cannot be agreed by the primary care teams at time of admission, or at any time during the inpatient stay, this must be **escalated to the PCH Executive on-call immediately**. The PCH Executive Director may consider and negotiate admission to an appropriate location (mental health unit or medical ward).
- An estimated discharge date (EDD) must be established and discussed with the family at admission.
- Family should be provided with relevant written information / [Hospital Brochure](#).

Admission to an Inpatient Unit

- Upon arrival at the ward, the treating team must undertake a concise but holistic assessment of the patient. Where possible the patient's parents / usual

caregivers should be involved in the assessment to establish baseline behaviours and usual means of communication, intellectual ability and sensory sensitivities.

- In considering cause of behavioural deterioration, medical, psychosocial and mental health assessments must be commenced on presentation.
- Referral to PCH KKIND Occupational Therapy for support with assessment and management of behaviour and self-regulation as well as planning for procedures that may cause increased anxiety and agitation. A Patient Profile will be developed to provide staff with important information about the patient's means of communication, behavioural triggers, positive behavioural supports and self-regulation strategies.

Medical considerations

- Investigate and exclude common causes of pain first:
 - infections (e.g. middle ear; urinary tract, chest, tonsillitis, sinus)
 - dental source
 - injury including fracture
 - gastrointestinal – e.g. constipation, appendicitis, peptic ulceration
 - current medications
 - seizure disorder
- Seek procedural support (may include pharmacological and non-pharmacological interventions) to facilitate medical or surgical examination or procedures.

Psychosocial considerations

- Obtain information from carers / community service providers regarding child's typical likes, dislikes, triggers, communication preferences, de-escalation strategies, sensory sensitivities and routines. Document in the patients individualised care plan, notes, or Patient Profile (where available).
- Engage the child / family's current social support agencies and contacts.
- If a situational crisis (rather than a physical or mental health issue) then liaise with the child / family's usual community providers, respite services and hospital social work.
 - Social work must be involved where admission length is anticipated > 24 hours

Mental Health considerations

- Consider emerging or deteriorating mental health disorder (e.g. anxiety, depression, agoraphobia) and consult with the mental health team as appropriate.

Medication Review and Management

- Where psychotropic medications are being considered for ongoing use, a review of the patient's current medication regime is warranted.
 - Refer to the Clinical Practice Manual [Agitation & Arousal Guideline](#)
 - Choice of medication and dosage may require further consideration if patient is on psychotropic medications prior to admission. Consider consultation with:
 - a psychiatrist
 - the child's usual community specialist/ private developmental clinician or usual prescriber
 - Head of Paediatrics, Child Development Service (CDS).

Positive Behaviour / De-escalation Techniques

- If available, obtain a copy of the child's Positive Behaviour Support Plan. Positive behaviour support strategies will be incorporated into the child's Patient Profile by the KKind Occupational Therapist.
- A non-judgemental attitude towards the behaviour of the child or adolescent is critical to gaining engagement.
- Where possible, adjust the child's environment to be consistent with their self-regulation requirements, for example a low stimulus room.
- Support engagement in meaningful occupation and normal daily routines

Transfer & Discharge Planning

- Patients anticipated to be in hospital for more than 24 hours are to be referred to the team social worker for early assessment and discharge planning.
- A multi-disciplinary team (MDT) meeting must be planned within 48 hours of admission for children who may require extended hospital stay and / or whose discharge requires complex planning.
 - The MDT is to formulate a discharge plan in collaboration with the family, carers, hospital clinicians and community care agencies, including the child's community paediatrician (CDS or private) and/or GP.
 - For prolonged admissions and complex discharges MDT meetings / case review to be held at least weekly.
 - Summary reports are to be prepared by the treating physician for PCH Executive Director or delegate.

Discharge

- A discharge summary must be completed and sent to the patient's community health care providers.

- Liaise (where relevant) with the agency who will be taking over care of child
- Debrief staff where necessary involving the Employee Assistance Program (EAP).

Escalation to Executive

- Patients deemed medically fit but unable to be discharged within 24 hours of the expected discharge date must be reported by the treating Consultant and/or Clinical Nurse Manager to the Co-Director/s of the respective medical or surgical division in the first instance, and escalated to the PCH Executive Director where applicable. Circumstances when this may arise include:
 - Family not in agreement with the discharge plan
 - Appropriate community placements are unavailable
- Co-Director or PCH Executive Director is to contact the relevant agency managers to facilitate discharge:

Department:	Who to contact:	How to contact:
Social work	PCH Head of Department	Via switch
Child Protection	PCH CPU Manager	Via switch
Perth Child Development Service	Paediatric Head of Department	1300 551 827 (in hours number)
DCPFS: Department for Child Protection and Family support	Local District Director closest to child's address or, Executive Director Metro or Country	Contact numbers available via the CPFS website
CAMHS	Head of Service, Ward 5A (PCH) On call psychiatrist after hours	Via switch
DSC: (Disabilities Service Commission) or NDIS <i>(Based on child's residential address)</i>	DSC Local Area Coordinator (LAC)	9426 9200 (general enquiries) <i>(LAC contact details available via the DSC website)</i>
	NDIS	1800 800 110
Other agencies as appropriate		

Related CAHS internal policies, procedures and guidelines
Agitation and Arousal Drug Management (Clinical Practice Manual)
Behavioural problems (PCH ED Guideline)
Discharge and Discharge Summary (PCH policy)
De-escalation (CAMHS Policy)
Engagement of Private Carer, Nurse or Therapist Policy
Nursing Care: Increased Level of or, Observation and/or Supervision
Oral Conscious Sedation
Restraint of Patients / Clients
References and related external legislation, policies, and guidelines
1. Scarpinato N, Bradley J, Kurbjun K, Bateman X, Holtzer B, and Ely B. Caring for the Child With an Autism Spectrum Disorder in the Acute Care Setting. J.Specialists Ped Nurs 15(3) 2010 p244-254
2. United Nations Convention on the Rights of the Child 1989. Article 23. Adopted by Australian Human Rights 1990. Available at: https://www.humanrights.gov.au/convention-rights-child
Useful resources
Autism Association of WA
Disabilities Service Commission – WA Local Coordination information & contacts

This document can be made available in alternative formats on request.

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